PATHXPERT LABS
A Unit of Amrutum Diagnostics Pvt. Ltd.

Foolmati Mata Mandir Ke Pass, Huzur
Rewa (M.P.) 486001
Mob. : 7024043326, 9109234253
E-mail : amrutumdiagnostics@gmail.com
Website : www.pathxpertlabs.com

PatientName	: **Mr.SANTOSH KUMAR**		
Age/Gender	: 50 Y/M	Collected	: 04/Oct/2025 07:10PM
Barcode No	: 10000717	Received	: 04/Oct/2025 07:13PM
Visit ID	: ADR1809	Reported	: 04/Oct/2025 08:07PM
Ref. By	: Dr.LAVKUSH PD. TIWARI	Status	: Final Report
UHID	: ADR.0000001808	Panel Name	: GASTRO WORLD & LIVER CENTER

DEPARTMENT OF HAEMATOLOGY

LP-1

COMPLETE BLOOD COUNT

Sample Type : WHOLE BLOOD EDTA

Test Name	Result	Unit	Bio. Ref. Range
RBC Parameters			
Haemoglobin (Hb) Non-Cyanmethemoglobin	13.7	g/dL	13.0-17.0
Red Blood Cell Count (TRBC) Electrical Impedence	4.77	10^6/µL	4.0-5.50
PCV/Haematocrit Cumulative Pulse	40.1	%	40.0-50.0
MCV Electrical Impedence	84.0	fL	80-100
MCH Electrical Impedence	28.7	pg	27-32
MCHC Electrical Impedence	34.2	gm/dl	32-36
RDW-CV Automatic Calculated	13.8	%	11.0-16.0
RDW-SD Calculated	44.2	fL	35.0-56.0
WBC Parameters			
Total Leucocyte Count (TLC) Electrical impedence and microscopy	**22.12**	10^3/uL	4.00 - 10.00
Differential Leucocyte Count			
Neutrophil	**88.9**	%	40-80
Lymphocyte	**7.1**	%	20-40
Monocyte	3.7	%	2-10
Eosinophil	**0.2**	%	1-6
Basophil	0.1	%	0-2
Absolute Leucocyte Counts			
Neutrophil	**19.67**	10^3/uL	2.00-7.00
Lymphocyte	1.57	10^3/uL	1.00-3.00
Monocyte	0.82	10^3/uL	0.20-1.00
Eosinophil	0.04	10^3/uL	0.02-0.50
Basophil	0.02	10^3/uL	0.00-0.10
Platelet Parameters			
Platelet Count Electrical Impedence & Microscopy	**146.00**	10^3/uL	150-450
Mean Platelet Volume(MPV) Automated Calculated	**15.8**	fL	6.5-12.0
Plateletcrit (PCT) Automated Calculated	**2.31**	%	0.12-0.37

Dr.Namita Pandey
MD Pathology,PDCC Oncopath
Consultant Pathologist

Foolmati Mata Mandir Ke Pass, Huzur
Rewa (M.P.) 486001
Mob. : 7024043326, 9109234253
E-mail : amrutumdiagnostics@gmail.com
Website : www.pathxpertlabs.com

PatientName	: **Mr.SANTOSH KUMAR**		
Age/Gender	: 50 Y/M	Collected	: 04/Oct/2025 07:10PM
Barcode No	: 10000717	Received	: 04/Oct/2025 07:13PM
Visit ID	: ADR1809	Reported	: 04/Oct/2025 08:07PM
Ref. By	: Dr.LAVKUSH PD. TIWARI	Status	: Final Report
UHID	: ADR.0000001808	Panel Name	: GASTRO WORLD & LIVER CENTER

DEPARTMENT OF HAEMATOLOGY

LP-1

Test Name	Result	Unit	Bio. Ref. Range
PDW-CV Automated Calculated	16.5	%	10.0-17.9
PDW-SD Automated Calculated	**28.0**	fL	9.0-17.0
P-LCR Calculated	**64.60**	%	18-50
P-LCC Calculated	94	10^9/L	44-140
Mentzer Index* Calculated	17.61	Ratio	<13

Interpretation :
The Complete Blood Count (CBC) provides an overall assessment of the hematological status including red blood cells, white blood cells, hemoglobin, hematocrit, indices, and platelet count. Correlation with clinical findings and other laboratory investigations is recommended for a definitive diagnosis.

***The Mentzer index (MI)** is a ratio of MCV to RBC count that helps differentiate iron deficiency anemia (IDA) from thalassemia trait. A MI > 13 suggests IDA. Conversely, an index < 13 suggests thalassemia trait. A value between 13 and 16 can be ambiguous and may require further testing, potentially indicating mixed anemia.

****Immature platelet fraction (IPF)**
An elevated IPF suggests increased platelet production, often seen in conditions like immune thrombocytopenic purpura (ITP) or during bone marrow recovery after chemotherapy. A low IPF, conversely, points to decreased platelet production within the bone marrow.

Dr.Namita Pandey
MD Pathology,PDCC Oncopath
Consultant Pathologist

PATHXPERT LABS
A Unit of Amrutum Diagnostics Pvt. Ltd.

Foolmati Mata Mandir Ke Pass, Huzur
Rewa (M.P.) 486001
Mob. : 7024043326, 9109234253
E-mail : amrutumdiagnostics@gmail.com
Website : www.pathxpertlabs.com

PatientName	: **Mr.SANTOSH KUMAR**		
Age/Gender	: 50 Y/M	Collected	: 04/Oct/2025 07:10PM
Barcode No	: 10000717	Received	: 04/Oct/2025 07:13PM
Visit ID	: ADR1809	Reported	: 04/Oct/2025 07:57PM
Ref. By	: Dr.LAVKUSH PD. TIWARI	Status	: Final Report
UHID	: ADR.0000001808	Panel Name	: GASTRO WORLD & LIVER CENTER

DEPARTMENT OF HAEMATOLOGY

LP-1

HBA1C (Glycosylated Hemoglobin)

Sample Type : WHOLE BLOOD EDTA

Test Name	Result	Unit	Bio. Ref. Range
GLYCOSYLATED HEMOGLOBIN (HPLC METHOD)	**6.6**	%	<5.7
ESTIMATED AVG. GLUCOSE	142.72	mg/dl	62.30-142.72

INTERPRETATION:
HbA1c ia used to monitor fluctuations in blood glucose conncentration in the past 8-12 weeks period.

As per American Diabetes Association (ADA) Guidelines

Reference Group	Non diabetic adults >=18 years	At risk (prediabetes)	Diagnosing Diabetes	Therapeutic goals for glycemic control
HbA1c in %	4.0 - 5.6	5.7-6.4	>=6.5	<7.0

Therapeutic Glycemic targets:-
Pregnant Diabetic Patients - Less than 6.5%
Children with type 1 Diabetes - Less than 7.0 %
Note: Presence of Hemoglobin variants and/or conditions that affect red cell turnover must be considered, particularly when the HbA1C result does not correlate with the patient's blood glucose levels.

Dr.Namita Pandey
MD Pathology,PDCC Oncopath
Consultant Pathologist

PATHXPERT LABS
A Unit of Amrutum Diagnostics Pvt. Ltd.

Foolmati Mata Mandir Ke Pass, Huzur
Rewa (M.P.) 486001
Mob. : 7024043326, 9109234253
E-mail : amrutumdiagnostics@gmail.com
Website : www.pathxpertlabs.com

PatientName	: **Mr.SANTOSH KUMAR**		
Age/Gender	: 50 Y/M	Collected	: 04/Oct/2025 07:10PM
Barcode No	: 10000718	Received	: 04/Oct/2025 07:13PM
Visit ID	: ADR1809	Reported	: 04/Oct/2025 09:20PM
Ref. By	: Dr.LAVKUSH PD. TIWARI	Status	: Final Report
UHID	: ADR.0000001808	Panel Name	: GASTRO WORLD & LIVER CENTER

DEPARTMENT OF BIOCHEMISTRY

LP-1

LIVER FUNCTION TEST (LFT)

Sample Type : SERUM

Test Name	Result	Unit	Bio. Ref. Range
Total Bilirubin DSA	0.80	mg/dL	0.20-1.20
Bilirubin Direct DSA	0.20	mg/dL	0.00-0.50
Bilirubin indirect Calculated	0.60	mg/dL	0.10-1.00
SGOT/AST IFCC	13.20	U/L	0.00-40.00
SGPT/ALT IFCC	17.20	U/L	0.00-40.00
SGOT/SGPT Ratio	0.77		
Alkaline Phosphatase IFCC	85.60	U/L	25.0-140.0
Total Protein Biuret	**6.30**	g/dL	6.60-8.30
Albumin BCG	4.10	g/dL	3.50 - 5.70
Globulin Calculated	**2.20**	g /dL	2.50-3.80
A/G Ratio Calculated	1.86	Ratio	1.30-2.10
GGT UV-assay	29.80	U/L	5-40

Interpretation :

The liver filters and processes blood as it circulates through the body. It metabolizes nutrients, detoxifies harmful substances, makes blood clotting proteins, and performs many other vital functions. The cells in the liver contain proteins called enzymes that drive these chemical reactions. When liver cells are damaged or destroyed, the enzymes in the cells leak out into the blood, where they can be measured by blood tests Liver tests check the blood for two main liver enzymes.

AST/SGOT : may indicate tissue/muscle injury.
ALT/SGPT : Almost exclusively found in the liver. If ALT and AST are found together in elevated amounts in the blood, liver damage is most likely present.
Alkaline Phosphatase and GGT: Linked to liver production and bile flow. When bile flow is slow or blocked, blood levels of these enzymes rise.
Proteins : Necessary for your body's growth, development, and health. Blood contains two classes of protein, albumin and globulin. High Protein levels 'Hyperproteinemia: May be seen in dehydration due to inadequate water intake or to excessive water loss (eg, severe vomiting, diarrhea, Addison's disease and diabetic acidosis) or as a result of increased production of proteins

PATHXPERT LABS
TRUST IN EVERY TEST

Dr.Namita Pandey
MD Pathology,PDCC Oncopath
Consultant Pathologist

PATHXPERT LABS
A Unit of Amrutum Diagnostics Pvt. Ltd.

Foolmati Mata Mandir Ke Pass, Huzur
Rewa (M.P.) 486001
Mob. : 7024043326, 9109234253
E-mail : amrutumdiagnostics@gmail.com
Website : www.pathxpertlabs.com

PatientName	: **Mr.SANTOSH KUMAR**		
Age/Gender	: 50 Y/M	Collected	: 04/Oct/2025 07:10PM
Barcode No	: 10000718	Received	: 04/Oct/2025 07:13PM
Visit ID	: ADR1809	Reported	: 04/Oct/2025 09:20PM
Ref. By	: Dr.LAVKUSH PD. TIWARI	Status	: Final Report
UHID	: ADR.0000001808	Panel Name	: GASTRO WORLD & LIVER CENTER

DEPARTMENT OF BIOCHEMISTRY

LP-1

KIDNEY FUNCTION TEST

Sample Type : SERUM

Test Name	Result	Unit	Bio. Ref. Range
Urea Urease GLDH	**51.70**	mg/dL	17.0-43.0
Blood Urea Nitrogen (BUN)	**24.16**	mg/dL	6.0-20.0
Creatinine Enzymatic	**2.40**	mg/dL	0.60-1.30
Estimated Glomerular Filtration Rate(eGFR)* CKD-EPI Creatinine equation 2021	**32.07**	mL/min/1.73sq m	Normal Or High:>=90~Mild Or Decrease:60-90~Mild To Moderate Decrease:45-59~Mild To Severe Decrease:30-44~Severe Decrease:15-29~Kidney Failure:<15
BUN/Creatinine Ratio	10.07	Ratio	0.0-23.0
Urea Creatinine Ratio	21.54	Ratio	0.0-45.0
Uric acid	6.90	mg/dl	3.5-7.2
Calcium Arsenazo	**8.30**	mg/dl	8.6-11.0
Phosphorus UV METHOD	**2.10**	mg/dL	2.5-5.0
Sodium ISE	**132.6**	mmol/L	135.0-145.0
Potassium ISE	4.3	mmol/L	3.5-5.0
Chloride ISE	95.4	mmol/L	95.0-109.0

Interpretation:

Blood urea nitrogen (BUN) and creatinine are waste products that are fltered out of the blood by the kidneys. Elevated levels of BUN and creatinine in the blood can indicate decreased kidney function.

The **glomerular fltration rate (GFR)** is a measure of how well your kidneys are fltering waste products from your blood. A low GFR can indicate decreased kidney function.

**** eGFR is applicable for patients aged 18 yrs or more.***

Dr.Namita Pandey
MD Pathology,PDCC Oncopath
Consultant Pathologist

PATHXPERT LABS
A Unit of Amrutum Diagnostics Pvt. Ltd.

Foolmati Mata Mandir Ke Pass, Huzur
Rewa (M.P.) 486001
Mob. : 7024043326, 9109234253
E-mail : amrutumdiagnostics@gmail.com
Website : www.pathxpertlabs.com

PatientName	: **Mr.SANTOSH KUMAR**		
Age/Gender	: 50 Y/M	Collected	: 04/Oct/2025 07:10PM
Barcode No	: 10000718	Received	: 04/Oct/2025 07:13PM
Visit ID	: ADR1809	Reported	: 04/Oct/2025 09:20PM
Ref. By	: Dr.LAVKUSH PD. TIWARI	Status	: Final Report
UHID	: ADR.0000001808	Panel Name	: GASTRO WORLD & LIVER CENTER

DEPARTMENT OF BIOCHEMISTRY

LP-1

LIPID PROFILE

Sample Type : SERUM

Test Name	Result	Unit	Bio. Ref. Range
Total Cholesterol CHOD-POD	**75.90**	mg/dL	<200
Triglycerides GPO-POD	**233.00**	mg/dL	<150
HDL Cholesterol Homogenous Direct Method	**20.00**	mg/dl	40-60
Non HDL Cholesterol Calculated	55.9	mg/dL	<100
LDL Cholesterol* Calculated/Direct	9.30	mg/dL	<100
VLDL Cholesterol** Calculated	**46.60**	mg/dL	<30
Chol/HDL Ratio Calculated	3.80	Ratio	3.5-5.0
LDL / HDL Ratio Calculated	0.47	Ratio	0.0-3.50
HDL/LDL Ratio Calculated	2.15	Ratio	0.5-3.0

* When triglyceride levels exceed 400 mg/dL, the standard Friedewald equation for calculating LDL cholesterol is unreliable and may underestimate true LDL-C levels, requiring a direct LDL-C measurement instead. This situation necessitates a direct, or calculated, LDL-C measurement to accurately assess cardiovascular risk, as the Friedewald method's accuracy declines significantly at high triglyceride concentrations.

** When triglyceride levels exceed 400 mg/dL, the standard Friedewald formula, which estimates VLDL cholesterol (VLDL-C) as 1/5th of the triglyceride level, becomes inaccurate.

Interpretation :

Lipid level assessments must be made following 9 to 12 hours of fasting, otherwise assay results might lead to erroneous interpretation. NCEP recommends of 3 different samples to be drawn at intervals of 1 week for harmonizing biological variables that might be encountered in single assays.

National Lipid Association Recommendations (NLA-2014)	Total Cholesterol (mg/dL)	Triglyceride (mg/dL)	LDL Cholesterol (mg/dL)	Non HDL Cholesterol (mg/dL)
Optimal	<200	<150	<100	<130
Above Optimal			100-129	130 - 159
Borderline High	200-239	150-199	130-159	160 - 189
High	>=240	200-499	160-189	190 - 219
Very High	-	>=500	>=190	>=220

HDL Cholesterol	
Low	High
<40	>=60

Risk Stratification for ASCVD (Atherosclerotic Cardiovascular Disease) by Lipid Association of India.

Dr.Namita Pandey
MD Pathology,PDCC Oncopath
Consultant Pathologist

Foolmati Mata Mandir Ke Pass, Huzur
Rewa (M.P.) 486001
Mob. : 7024043326, 9109234253
E-mail : amrutumdiagnostics@gmail.com
Website : www.pathxpertlabs.com

PatientName	: **Mr.SANTOSH KUMAR**		
Age/Gender	: 50 Y/M	Collected	: 04/Oct/2025 07:10PM
Barcode No	: 10000718	Received	: 04/Oct/2025 07:13PM
Visit ID	: ADR1809	Reported	: 04/Oct/2025 09:20PM
Ref. By	: Dr.LAVKUSH PD. TIWARI	Status	: Final Report
UHID	: ADR.0000001808	Panel Name	: GASTRO WORLD & LIVER CENTER

DEPARTMENT OF BIOCHEMISTRY

LP-1

Test Name	Result	Unit	Bio. Ref. Range
Risk Category	A. CAD with > 1 feature of high risk group		
Extreme risk group	B. CAD with >1 feature of very high risk group of recurrent ACS (within 1 year) despite LDL-C <or = 50 mg/dl or poly vascular disease		
Very High Risk	1.Established ASCVD 2.Diabetes with 2 major risk factors of evidence of end organ damage 3. Familial Homozygous Hypercholesterolemia		
High Risk	1. Three major ASCVD risk factors 2. Diabetes with 1 major risk factor or no evidenceof end organ damage 3. CHD stage 3B or 4. 4 LDL >190 mg/dl 5. Extreme of a single risk factor 6. Coronary Artery Calcium - CAC > 300 AU 7. Lipoprotein a >/= 50 mg/dl 8. Non stenotic carotid plaque		
Moderate Risk	2 major ASCVD risk factors		
Low Risk	0-1 major ASCVD risk factors		
Major ASCVD (Atherosclerotic cardiovascular disease) Risk Factors			
1. Age >/=45 years in Males & >/= 55 years in Females	3. Current Cigarette smoking or tobacco use		
2. Family history of premature ASCVD	4. High blood pressure		
5. Low HDL			

Newer treatment goals and statin initiation thresholds based on the risk categories proposed by Lipid Association of India in 2020.

Risk Group	Treatment Goals		Consider Drug Therapy	
	LDL-C (mg/dl)	Non-HDL (mg/dl)	LDL-C (mg/dl)	Non-HDL (mg/dl)
Extreme Risk Group Category A	<50 (Optional goal <OR = 30)	<80 (Optional goal <OR = 60)	>OR = 50	>OR = 80
Extreme Risk Group Category B	>OR = 30	>OR = 60	> 30	> 60
Very High Risk	<50	<80	>OR = 50	>OR = 80
High Risk	<70	<100	>OR = 70	>OR = 100
Moderate Risk	<100	<130	>OR = 100	>OR = 130
Low Risk	<100	<130	>OR = 130*	>OR = 160

*** After an adequate non-pharmacological intervention for at least 3 months.**

References : Management of Dyslipidaemia for the Prevention of Stroke : Clinical practice Recommendations from the Lipid Association of India. Current Vascular Pharmacology,2022,20,134-155.

Dr.Namita Pandey
MD Pathology,PDCC Oncopath
Consultant Pathologist

PATHXPERT LABS
A Unit of Amrutum Diagnostics Pvt. Ltd.

Foolmati Mata Mandir Ke Pass, Huzur
Rewa (M.P.) 486001
Mob. : 7024043326, 9109234253
E-mail : amrutumdiagnostics@gmail.com
Website : www.pathxpertlabs.com

PatientName	: **Mr.SANTOSH KUMAR**		
Age/Gender	: 50 Y/M	Collected	: 04/Oct/2025 08:03PM
Barcode No	: 10000718	Received	: 04/Oct/2025 08:04PM
Visit ID	: ADR1809	Reported	: 04/Oct/2025 09:20PM
Ref. By	: Dr.LAVKUSH PD. TIWARI	Status	: Final Report
UHID	: ADR.0000001808	Panel Name	: GASTRO WORLD & LIVER CENTER

DEPARTMENT OF BIOCHEMISTRY

C-reactive protein

Test Name	Result	Unit	Bio. Ref. Range
C-REACTIVE PROTEIN TURBIDIMETRY	**127.30**	mg/L	<6.0 mg/L

INTERPRETATION:
CRP is one of the proteins commonly referred to as acute phase reactants. CRP is distinguished by its rapid response to trauma or infection. CRP levels returns to normal quickly at the end of the acute episode. Measurement of CRP aids in evaluation of the amount of injury to body tissues.

Dr.Namita Pandey
MD Pathology,PDCC Oncopath
Consultant Pathologist

PATHXPERT LABS
A Unit of Amrutum Diagnostics Pvt. Ltd.

Foolmati Mata Mandir Ke Pass, Huzur
Rewa (M.P.) 486001
Mob. : 7024043326, 9109234253
E-mail : amrutumdiagnostics@gmail.com
Website : www.pathxpertlabs.com

PatientName	: **Mr.SANTOSH KUMAR**		
Age/Gender	: 50 Y/M	Collected	: 04/Oct/2025 07:10PM
Barcode No	: 10000718	Received	: 04/Oct/2025 07:13PM
Visit ID	: ADR1809	Reported	: 04/Oct/2025 08:23PM
Ref. By	: Dr.LAVKUSH PD. TIWARI	Status	: Final Report
UHID	: ADR.0000001808	Panel Name	: GASTRO WORLD & LIVER CENTER

DEPARTMENT OF HORMONE ASSAYS

LP-1

Test Name	Result	Unit	Bio. Ref. Range
FERRITIN			
Sample Type : SERUM			
Ferritin CLIA	150.6	ng/mL	25-350

Interpretation :

Ferritin is the storage form of iron, remains in the body tissues until it is needed for erythropoiesis.

Low serum ferritin : Best laboratory indicator of iron depletion. Serum ferritin is clinically useful in distinguishing between iron-deficiency anemia (serum ferritin levels diminished) and anemia of chronic disease(serum ferritin levels usually normal or elevated). Also, a good screening test in distinguishing iron deficiency (low values) and thalassemia minor (normal or high values).
Causes for low Ferritin levels: Decreased iron intake, poor diet, low gastro-interstinal absorption due to crohn's diesease or celiac dieseas, hemorrhoids, colon Carcinoma, peptic ulcer, menorrhagia, hematuria.

High serum ferritin : Seen in hemochromatosis and other iron-overload states, as well as acute hepatitis, Gaucher disease,malignancies and chronic inflammatory disorders. . Ferritin is also an acute phase reactant.

Dr.Namita Pandey
MD Pathology,PDCC Oncopath
Consultant Pathologist

Foolmati Mata Mandir Ke Pass, Huzur
Rewa (M.P.) 486001
Mob. : 7024043326, 9109234253
E-mail : amrutumdiagnostics@gmail.com
Website : www.pathxpertlabs.com

PatientName	: **Mr.SANTOSH KUMAR**		
Age/Gender	: 50 Y/M	Collected	: 04/Oct/2025 07:10PM
Barcode No	: 10000718	Received	: 04/Oct/2025 07:13PM
Visit ID	: ADR1809	Reported	: 04/Oct/2025 08:23PM
Ref. By	: Dr.LAVKUSH PD. TIWARI	Status	: Final Report
UHID	: ADR.0000001808	Panel Name	: GASTRO WORLD & LIVER CENTER

DEPARTMENT OF HORMONE ASSAYS

LP-1

Test Name	Result	Unit	Bio. Ref. Range

25-OH VITAMIN D

Sample Type : SERUM

25 - OH Vitamin D CLIA	35.41	ng/mL	30-120

NOTE : 25-hydroxy vitamin D is the main circulating form of Vitamin D in the blood, reflecting overall Vitamin D status from sun exposure, diet and supplements. It measures both metabolites D2 (Ergocalciferol) and D3 (Cholecalciferol). A blood test is used to diagnose deficiency or to monitor levels in people taking supplements.

INTERPRETATION :
1. Deficiency : <20 ng/ml.
2. Insufficiency : 20-30 ng/ml.
3. Sufficiency :30-100 ng/ml.
4. Toxicity : > 100 ng/ml

Decreased levels of 25-OH Vitamin D appear in patient with:
- Nutritional rickets / osteomalacia
- Senile / post menopausal osteoporosis
- Celiac disease
- Inflammatory bowel disease (e.g. Crohn"s disease)
- Insufficiency of the exocrine pancreas
- Short bowel syndrome
- Billary cirrhosis, liver dysfunction
- Renal osteodystrophy
- Nephrotic syndrome
- Neonatal hypocalcemia
- Hypocalcemia
- Treatment with anticonvulsant drugs (enhanced metabolism) .

Dr.Namita Pandey
MD Pathology,PDCC Oncopath
Consultant Pathologist

PATHXPERT LABS
A Unit of Amrutum Diagnostics Pvt. Ltd.

Foolmati Mata Mandir Ke Pass, Huzur
Rewa (M.P.) 486001
Mob. : 7024043326, 9109234253
E-mail : amrutumdiagnostics@gmail.com
Website : www.pathxpertlabs.com

PatientName	: **Mr.SANTOSH KUMAR**		
Age/Gender	: 50 Y/M	Collected	: 04/Oct/2025 07:10PM
Barcode No	: 10000718	Received	: 04/Oct/2025 07:13PM
Visit ID	: ADR1809	Reported	: 04/Oct/2025 08:29PM
Ref. By	: Dr.LAVKUSH PD. TIWARI	Status	: Final Report
UHID	: ADR.0000001808	Panel Name	: GASTRO WORLD & LIVER CENTER

DEPARTMENT OF HORMONE ASSAYS

LP-1

VITAMIN B12

Test Name	Result	Unit	Bio. Ref. Range
Vitamin B12 CLIA	268.11	pg/mL	211-911

COMMENTS:
Vitamin B12 belongs to the family of cobalamins and serves as a cofactor for two important reactions in humans. As methylcobalamin, it is a cofactor for methionine synthetase in the conversion of homocysteine to methionine, and as adenosylcobalamin for the conversion of methylmalonyl-coenzyme A (CoA) to succinyl-CoA. All vitamin B12 comes from diet and is present in all foods of animal origin. Severe prolonged vitamin B12 deficiency may cause megaloblastic anemia and/or neurological degeneration.

- Causes of vitamin B12 deficiency
- Dietary deficiency (strict vegetarians)
- Decreased production of intrinsic factor
- Helicobacter pylori infection
- Competition for vitamin 812 in gut
- Blind loop syndrome
- Decreased ileal absorption of vitamin 812
- Pancreatic insufficiency
- Surgical resection

Dr.Namita Pandey
MD Pathology,PDCC Oncopath
Consultant Pathologist

Foolmati Mata Mandir Ke Pass, Huzur
Rewa (M.P.) 486001
Mob. : 7024043326, 9109234253
E-mail : amrutumdiagnostics@gmail.com
Website : www.pathxpertlabs.com

PatientName	: **Mr.SANTOSH KUMAR**		
Age/Gender	: 50 Y/M	Collected	: 04/Oct/2025 07:10PM
Barcode No	: 10000718	Received	: 04/Oct/2025 07:13PM
Visit ID	: ADR1809	Reported	: 04/Oct/2025 08:08PM
Ref. By	: Dr.LAVKUSH PD. TIWARI	Status	: Final Report
UHID	: ADR.0000001808	Panel Name	: GASTRO WORLD & LIVER CENTER

DEPARTMENT OF HORMONE ASSAYS

LP-1

Test Name	Result	Unit	Bio. Ref. Range
Thyroid stimulating hormone (TSH)			
Sample Type : SERUM			
THYROID STIMULATING HORMONE [TSH] CLIA	3.792	µIU/mL	0.35 - 4.95

Note

1. TSH levels are subject to circadian variation, reaching peak levels between 2 - 4.a.m. and at a minimum between 6-10 pm . The variation is of the order of 50%, hence time of the day has influence on the measured serum TSH concentrations
2. Values <0.03 µIU/mL need to be clinically correlated due to presence of a rare TSH variant in some individuals.
3. Transient increase in TSH levels or abnormal TSH levels can be seen in various nonthyroidal diseases. Simultaneous measurement of TSH with free T4 is useful in evaluating the differential diagnosis

CAUTIONS

Sick, hospitalized patients may have falsely low or transiently elevated thyroid stimulating hormone. Some patients who have been exposed to animal antigens, either in the environment or as part of treatment or imaging procedure, may have circulating antianimal antibodies present. These antibodies may interfere with the assay reagents to produce unreliable results.

Ultra Sensitive TSH ref range in Pregnacy Reference range (microIU/ml)

First triemester 0.24 - 2.00
Second triemester 0.43-2.2
Third triemester 0.8-2.5

Dr.Namita Pandey
MD Pathology,PDCC Oncopath
Consultant Pathologist

PATHXPERT LABS
A Unit of Amrutum Diagnostics Pvt. Ltd.

Foolmati Mata Mandir Ke Pass, Huzur
Rewa (M.P.) 486001
Mob. : 7024043326, 9109234253
E-mail : amrutumdiagnostics@gmail.com
Website : www.pathxpertlabs.com

PatientName	: **Mr.SANTOSH KUMAR**		
Age/Gender	: 50 Y/M	Collected	: 04/Oct/2025 07:22PM
Barcode No	: 10000718	Received	: 04/Oct/2025 07:22PM
Visit ID	: ADR1809	Reported	: 04/Oct/2025 07:37PM
Ref. By	: Dr.LAVKUSH PD. TIWARI	Status	: Final Report
UHID	: ADR.0000001808	Panel Name	: GASTRO WORLD & LIVER CENTER

DEPARTMENT OF HORMONE ASSAYS

Test Name	Result	Unit	Bio. Ref. Range
TROPONIN - T			
Sample Type : SERUM			
TROPONIN T Rapid Test	Negative		Negative

COMMENTS:
Cardiac Troponin T elevates in all patients with acute myocardial infarction (MI). Clinical findings, ECG & other serological findings should be considered for a confirmative diagnosis. The initial appearance of elevated cTnT in the blood of individuals suffered from AMI is variable; from 3-24 hours depending on factors such as infarct size. Elevated cTnT levels can be measured for up to 14 days in some patients.

LIMITATIONS:
Erroneous findings may be obtained from samples taken from patients who have been treated with monoclonal mouse antibodies or have received them for diagnostic purposes. Patients with myocardial injury, unstable angina, myocarditis, cardiac contusions and heart transplants shows elevation of troponin T. Elevations also can be seen in selected patients with rhabdomyolysis, polymyositis and renal disease. Due to the release Kinetics of troponin T, a negative result within the first hours of the onset of symptoms does not rule out myocardial infarction.

Dr.Namita Pandey
MD Pathology,PDCC Oncopath
Consultant Pathologist

Foolmati Mata Mandir Ke Pass, Huzur
Rewa (M.P.) 486001
Mob. : 7024043326, 9109234253
E-mail : amrutumdiagnostics@gmail.com
Website : www.pathxpertlabs.com

PatientName	: **Mr.SANTOSH KUMAR**		
Age/Gender	: 50 Y/M	Collected	: 04/Oct/2025 08:03PM
Barcode No	: 10000718	Received	: 04/Oct/2025 08:04PM
Visit ID	: ADR1809	Reported	: 04/Oct/2025 08:42PM
Ref. By	: Dr.LAVKUSH PD. TIWARI	Status	: Final Report
UHID	: ADR.0000001808	Panel Name	: GASTRO WORLD & LIVER CENTER

DEPARTMENT OF IMMUNOLOGY

Test Name	Result	Unit	Bio. Ref. Range
PROCALCITONIN			
Sample Type : SERUM			
Procalcitonin	**55.64**	ng/ml	0.-0.50

Interpretation:-
Note : For diagnostic purpose,the result should always be interpreted in conjunction with patients medical history as several noninfectious causes can also elevate Procalcitonin levels

Comments:-
Procalcitonin is a prohormone expressed by neuroendocrine cells(C cells of thyroid, pulmonary and pancreatic tissues) present in very low levels in healthy individuals. Measurement of PCT can be used as a marker of severe sepsis and generally grades well with the degree of sepsis . Evidence is emerging that PCT levels can reduce unnecessary antibiotic prescribing in lower respiratory tract infections.

Increased Levels :-·
Infectious - Bacterial infections leading to Septic shock/Severe sepsis, Community acquired respiratory tract infections & Ventilator induced pneumonia
Non Infectious - Severe Cardiogenic shock, organ perfusion anomalies, Small cell lung cancer or Medullary C-cell carcinoma of thyroid, major trauma, surgical intervention & severe burns, treatment which stimulate the release of pro-inflammatory cytokines & Neonates (<48 hrs. after birth)
Clinical Use
· As a prognostic marker to support outcome prediction in sepsis patients. · As an indicator of severity and major complications in acute pancreatitis.
· As a guide for the necessity of antibiotic therapy and to monitor success of treatments in patients
Please Correlate with Clinical Condition.

Dr.Namita Pandey
MD Pathology,PDCC Oncopath
Consultant Pathologist

PATHXPERT LABS
A Unit of Amrutum Diagnostics Pvt. Ltd.

Foolmati Mata Mandir Ke Pass, Huzur
Rewa (M.P.) 486001
Mob. : 7024043326, 9109234253
E-mail : amrutumdiagnostics@gmail.com
Website : www.pathxpertlabs.com

PatientName	: **Mr.SANTOSH KUMAR**		
Age/Gender	: 50 Y/M	Collected	: 04/Oct/2025 07:10PM
Barcode No	: 10000719	Received	: 04/Oct/2025 07:13PM
Visit ID	: ADR1809	Reported	: 04/Oct/2025 07:55PM
Ref. By	: Dr.LAVKUSH PD. TIWARI	Status	: Final Report
UHID	: ADR.0000001808	Panel Name	: GASTRO WORLD & LIVER CENTER

DEPARTMENT OF CLINICAL PATHOLOGY

LP-1

URINE ROUTINE EXAMINATION

Sample Type : URINE

Test Name	Result	Unit	Bio. Ref. Range
GROSS EXAMINATION			
Quantity	25	ml	-
Colour	Pale Yellow	-	Pale Yellow
Appearance	Slightly Hazy	-	Clear
Deposit	Absent	-	Absent
CHEMICAL EXAMINATION			
pH Double Indicator	6.00	-	5-8
Specific Gravity Ion Exchange Chromatography	1.010	-	1.010-1.035
Glucose Glucose-oxidase/Peroxidase reaction	**Positive (++)**		Negative
Protein Protein - error of Indicators	**Positive (+)**		Negative
Microalbumin * Spot Urine,Immunoturbidimetric	>0.15	g/L	<0.15
Ketone Bodies Nitroprusside	Negative		Negative
Urobilinogen Modified Ehrlichs Reaction	Normal		Normal
Bilirubin Azo-coupling Reaction	Negative		Negative
Blood Peroxidase	**Positive (Trace)**	-	Negative
Leucocyte Azo-coupling reaction	**Positive (++)**	-	Negative
Nitrite Diazotization Reaction	**Positive**	-	Negative
MICROSCOPIC EXAMINATION			
Pus Cells	**25-30**	/hpf	0-5
Epithelial Cells	2-3	/hpf	0-5
R . B . C	**4-5**	/hpf	0-2
Casts	Absent	/hpf	Absent
Crystals	Absent		Absent
Bacteria	**Present**	-	Absent
Yeast Cells	Absent	-	Absent

Confirmative Testing: Because the test is semi-quantitative, a positive or high result often leads to more precise laboratory testing to confirm and measure the exact level of specific parameters.

Namita

Dr.Namita Pandey
MD Pathology,PDCC Oncopath
Consultant Pathologist

PATHXPERT LABS
A Unit of Amrutum Diagnostics Pvt. Ltd.

Foolmati Mata Mandir Ke Pass, Huzur
Rewa (M.P.) 486001
Mob. : 7024043326, 9109234253
E-mail : amrutumdiagnostics@gmail.com
Website : www.pathxpertlabs.com

PatientName	: **Mr.SANTOSH KUMAR**		
Age/Gender	: 50 Y/M	Collected	: 04/Oct/2025 07:10PM
Barcode No	: 10000719	Received	: 04/Oct/2025 07:13PM
Visit ID	: ADR1809	Reported	: 04/Oct/2025 07:55PM
Ref. By	: Dr.LAVKUSH PD. TIWARI	Status	: Final Report
UHID	: ADR.0000001808	Panel Name	: GASTRO WORLD & LIVER CENTER

DEPARTMENT OF CLINICAL PATHOLOGY

LP-1

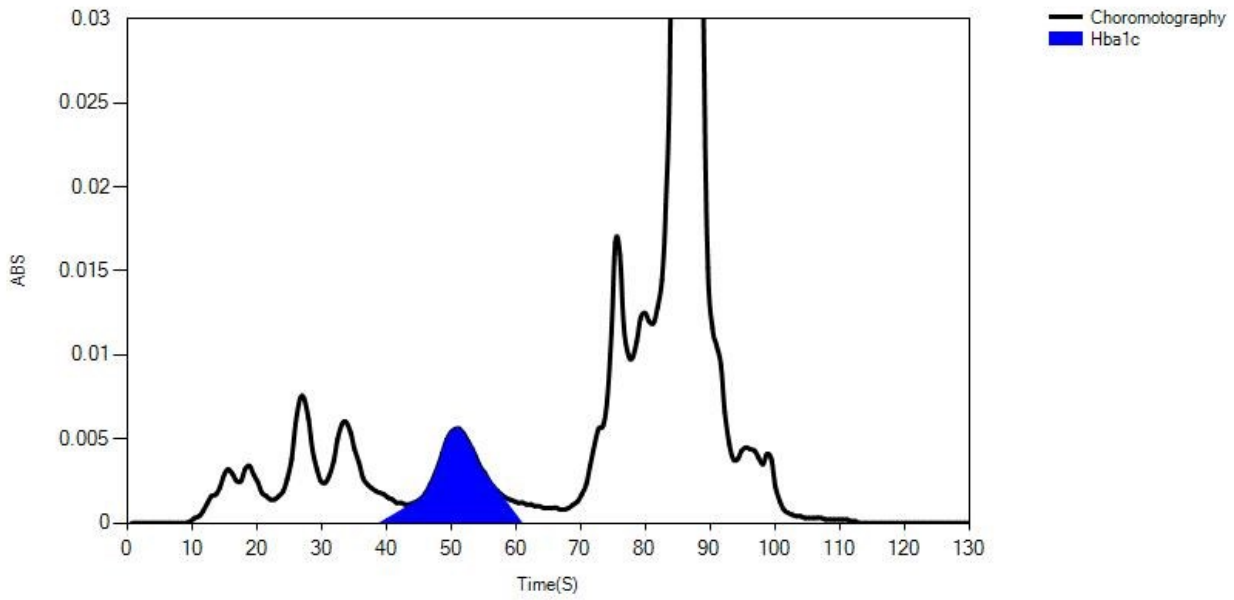
Test Name	Result	Unit	Bio. Ref. Range

***Please Correlate Clinically**

***** End Of Report *****

Dr.Namita Pandey
MD Pathology,PDCC Oncopath
Consultant Pathologist

LIFOTRONIC Graph Report

Name :	Case :	Patient Type :		Test Date :	04/10/2025 19:41:55
Age :	Department :	Sample Type :	Whole Blood EDTA	Sample Id :	10000717
Gender :				Total Area :	10451

Peak Name	Retention Time(s)	Absorbance	Area	Result (Area %)
HbA0	86	2299	8895	85.1
HbA1c	50	56	625	6.6
La1c	33	61	367	3.9
HbF	26	76	312	3.3
Hba1b	18	34	125	1.3
Hba1a	15	32	127	1.3